



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صباح سليمان بشاي يونس
المحافظة : القاهرة
الحى / المدينة : المعصره
رقم البطاقة : 26303212102246 رقم التليفون : 01208285370
العمر (تاريخ الميلاد) : 21-03-1963 المهنة : ربه منزل
العنوان : زهراء حلوان
تاريخ تسجيل الإستمارة : 29-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي بيمبروليزوماب وربدي [14335] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة الأبحاث المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: صباح سليمان بشاي يونس
National ID: 26303212102246



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المريضة تعاني من ورم غير حميد بالبدن و تحتاج الي Pembrolizumab IV

الوقت :

---:---

التاريخ :

05-07-2026

الطبيب المعالج :

د/شيماء لاثنين



٢٠٢٥/٠٨ ٢٦٣.٣٢١٢١.٠٢٢٤٦
حاصلة على دبلوم معهد فنى تجارى



أنثى مسيحية متزوجة
عماد شفيق ندا ابراهيم
البطاقة سارية حتى ٢٠٢٢/٠٨/١٧



جمهورية مصر العربية
بطاقة تحليق شخصية



صباح
سليمان بشاى يونس

١٢ ش ١ - زهراء حنون
المعصرة - القاهرة



٢٦٣.٣٢١٢١.٠٢٢٤٦

LH6678911

05/12/2016

MDT FORM

Name:	Sabah Al-Sabah	MRN:	9706
Age:	63 Y	ID:	25353232102245
Menopausal Status:	POST	Phone:	01221447008
NRS SCORE:	PS 0	Marital Status:	MARRIED
BMI:	20	Family History:	+VE
CO Morbidity:	DIABETIC	Reason Of MDT:	NEW CASE

Clinical Data:

BX: 101 G3
ER LUS PR 0/0 HER +1 K167 30%

R

L

Sonomammography:

ACR C, RT UD CENTRAL MASS 2.3*2.7, 2 CM FROM NIPPLE 5 MM DEEP TO SKIN.
ANT DUCTAL EXTENSION 2MM IN CALIBER.
NON-SPECIFIC IWS

Mets Workup:

CT CAP WITH CONTRAST - FREE
BONE SCAN - FREE

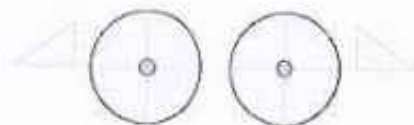
History:

Recommendations:

FOR POSSIBLE CLIPPING, PORTA CATH INSERTION,
NACT THEN RT CENTRAL Q/ADENECTOMY AND

MDT Outcome & Plan

For:
1. CSM and if unilateral Clip insertion
2. BRCA testing
3. Echo
4. Portacath insertion
5. Then start NACT Keynote 522, Eprex/Carbo x12
then AC x4 with concomitant Rebolizumab.
Re-MDT after CSM for surgical decision.



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذ / صباح سليمان بشاي	Request No.	: 13005396
Patient No.	: 26303212102246	Request Date	: 10/03/2026
Path No.	: 671/26	Verification Date	: 16/03/2026
Sex	: Female	Financial Category	: جوماتاف روسي - ميداء
Age	: 62 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast 9 O'clock mass, (BIRADs 4c).

Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 1:20 pm, Cold ischemia time: 20 seconds.

Gross Examination:

- Three fragmented grey white tissue cores, ranging in size between 0.3 cm & 1.5 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.

Diagnosis:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghonaim.

Verified by :

Dr. Duaa Mohamed

Printing Date : 28/03/2026 10:07:52

Page 1 of 1

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / صباح سليمان بشاي	Request No. : 13005396
Patient No. : 26303212102246	Request Date : 10/03/2026
Path No. : 671/26	Verification Date : 16/03/2026
Sex : Female	Financial Category : جوماتاف روسي - مبالره
Age : 62 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast 9 O'clock mass, (BIRADs 4c) diagnosed "Invasive duct carcinoma grade 3".
Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 1:20 pm, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 60%.

Conclusion:

Right breast mass, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Low (score 1+).
- Ki67%: 60%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 56% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2,

Verified by :

Dr.Duaa Mohamed

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Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذة / صدياح سليمان بشاي
Patient No. 26303212102246
Path No. 671/26
Sex Female
Age 62 Y
Referring Doc.

Request No. 13005396
Request Date 10/03/2026
Verification Date 16/03/2026
Financial Category جوشاف روسي - ميلارد
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim.

Verified by :

Dr. Duaa Mohamed

Printing Date : 28/03/2026 10:07:59

Page 2 of 2

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qalema - Cairo, Egypt

Patient's Name: SABAH SOLIMAN BOSHAY BOLIS
Date: 2026-04-06
Referred by: Prof.Dr. CT CAP IV
Hospital: Goustave Roussey International Egypt

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH
CORONAL RECONSTRUCTION: -**

Findings:

- Right breast soft tissue mass measures 17x15 mm
- Few right pectoral lymph nodes measures 11x9 mm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

Right breast soft tissue mass for US correlation.

**MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH
ORAL & I.V. CONTRAST ADMINISTRATION: -**

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.

Patient's Name: SABAH SOLIMAN BOSHAY BOLIS
Date: 2026-04-06
Referred by: Prof.Dr. CT CAP IV
Hospital: Goustave Roussey International Egypt

- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- ***Normal CT scan of the abdomen and pelvis.***

Dr Mai Mohamed MD

Dr.Mai Sadiq

Dr.Mai Sadiq



S C A N

Name: Sabah Soliman Beshay

ID: 5140974

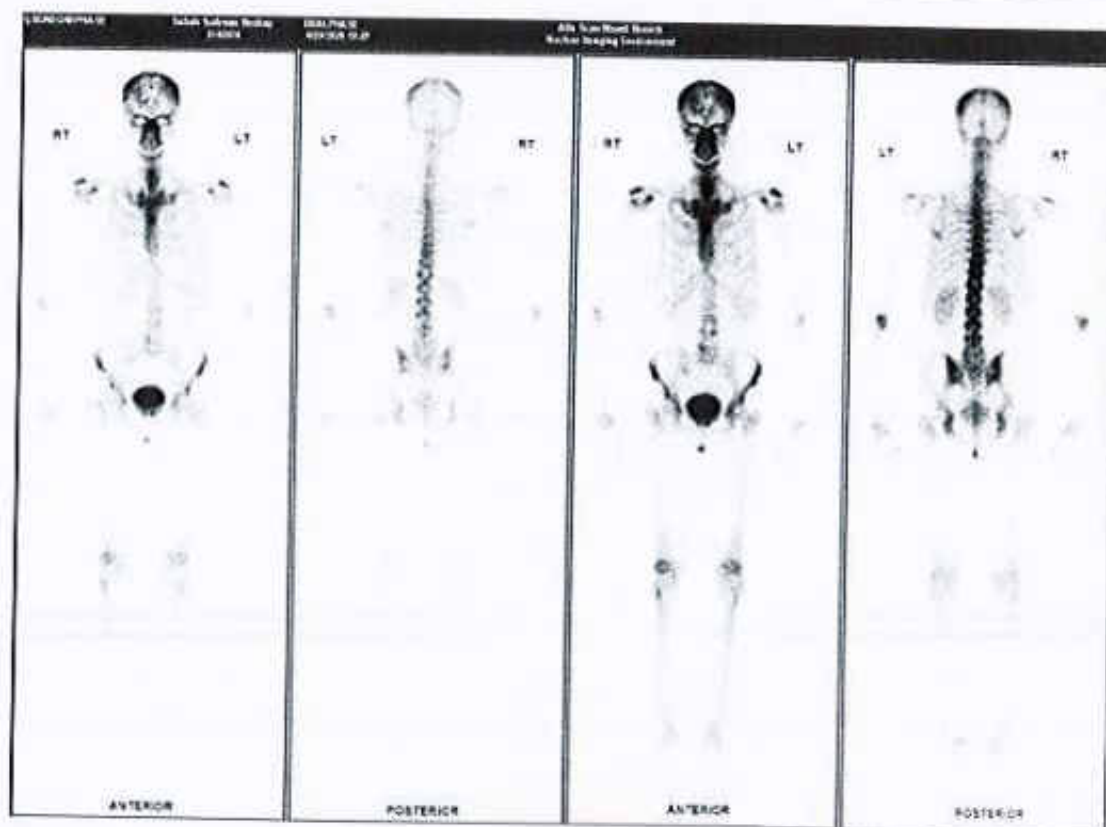
Date: 20/04/2026

^{99m}Tc-MDP wholebody Bone Scan**Clinical data:**

Indication	Right breast cancer.
Tumor markers	No.
Skeletal trauma/ surgeries	No.
Previous imaging	Not available.
Previous bone scan	No.

Technique:

Injected dose	740 MBq
Images	Delayed whole body +spot images of the skull and chest.
Complementary images	No.





^S_C^A_N
Findings:

- No abnormal osseous tracer uptake detected that would account for osseous lesions.
- Arthritic changes of both shoulders are noted.

Impression:

- No scintigraphic evidence of metastatic osseous deposits.

Dr. Mona Saied, MD

Dr. Omnia Alaa, MSc

ID: 5140974

الفاسكان

AMG
ALFA MEDICAL GROUP

A MEMBER OF ALFA MEDICAL GROUP

Bone Scan

16171

[DOCTYPESBONE_SCAN]

التاريخ	2026/6/14	Sabah Soliman	63 السن
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Echo report

LVDd	3.5	4.2-5.8cm	FS	25	25-43%
LVSd	2.4	2.5-4cm	EF	58	>55%
IVSd	0.7	0.7-1	AO	2.8	2.6-3.0cm
LVPWd	0.8	0.7-1	LA	3.6	3-4cm

Measurements:

Normal LV dimensions with good systolic function by MM.

- Normal LV wall thickness
- SWMA: Not apparent
- RV wall dimensions, wall thickness, function, TAPSE good
- Aortic root: normal
- LA size: normal
- RA size: Normal
- Septae: Normal

Valves:

- Normal AV morphology and flow
- Normal MV and flow
- Normal TV and flow
- Normal PV and flow

Pericardium: normal

Intracardiac masses: no

Conclusion: Diastolic dysfunction

**Dr Ahmed Maher Msc
Cardiology**

Patient ID: 90714	Patient Name: English, Sarah Selman Bushay
Age: 63	25/06/2024 02:54 PM Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: 63-year old female with positive family history (paternal side grandmother) complaining of right mastalgia.

Technique

Medio-lateral oblique and CC views. High resolution ultrasound

Detailed Findings

Right breast

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR c).
- Outer central, middle third, irregular speculated high density mass lesion with obscured margins and related coarse heterogeneous and linear microcalcifications.
- Ultrasound demonstrated exaggerated fibroglandular pattern for age with heterogeneous parenchyma showing mottled appearance.
- 9 o'clock, middle third, 3 cm from nipple, 6 mm deep to the skin, irregular anti-parallel heterogeneous hypochoic solid mass lesion with moderate internal vascularity measuring 23 x 27 mm.
- Anterior ductal extension is seen reaching the nipple through segmental prominent soft tissue echogenicity filled ducts seen reaching 3 mm in caliber.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR c).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated exaggerated fibroglandular pattern for age with heterogeneous parenchyma showing mottled appearance with no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Right breast (BIRADS 4c): Outer central suspicious mass lesion with anterior ductal extension reaching the nipple. US-Guided Tru-cut biopsy was taken at same setting.

Left breast (BIRADS 1): Diffuse breast adenosis.

Recommendations:

For histopathological correlation.

Dr. Amal Hozzei, M.Sc.

Q.N.7

**GUSTAVE
ROUSSY**
HOSPITAL

Patient's Name:
Date:
Referred by:
Referral:

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2026-04-06
Prof. Dr. CT CAP IV
Gustave Roussy International Egypt

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- Smooth filling of the urinary bladder with no filling defects or diverticula outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- **Normal CT scan of the abdomen and pelvis.**

Dr Mai Mohamed MD

Dr. Mai Sadiq

Dr. Mai Sadiq

GUSTAVE
ROUSSY

Patient's Name:
DOB:
Referral by:
Referral:

SADAB SOLEMAN HUSSEIN HUSSEIN
2024-04-06
2024-04-06
Gustave Roussy International Egypt

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**GUSTAVE
ROUSSY**

Patient's Name:
Date:
Referred by:
Hospital:

SABER MOHAMMED HANAN HUSSEIN
1976-01-06
Prof. Dr. F. E. A. A. A. A.
Gustave Roussy International Egypt

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH
CORONAL RECONSTRUCTION: -**

Findings:

- Right breast soft tissue mass measures 12x15 mm.
- Right axillary lymph nodes measures 11x9 mm.
- Clear both lung fields, no sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
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**GUSTAVE
ROUSSY**
INTERNATIONAL
EGYPT

Patient's Name:
Date:
Referred by:
Hospital:

SABAH SOLIMAN BOSHAY BOLIS
2026-04-06
Profile: CT CAP IV
Gustave Roussy International Egypt

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Opinion:

- **Normal CT scan of the abdomen and pelvis.**

Dr Mai Mohamed MD

Dr. Mai Sadiq

Dr. Mai Sadiq

PATIENT NAME صباح سليمان بشاي		Registered 08-06-2026 17:23:08	Collected 08-06-2026 17:24:19
Visit Number 50726508318	Age 63 Year	Authenticated 07-07-2026 15:08:47	Printed 14-07-2026 10:02:46
Gender Female		Referred By Prof : -	Client ID 55452

MOLECULAR BIOLOGY REPORT

BRCA 1.2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result : No Pathogenic variants detected in *BRCA1* or *BRCA2* genes sequencing.

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the *BRCA1* & *BRCA2* genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.

- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.

- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, *BRCA2* carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay

- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of *BRCA1* & *BRCA2* genes.

- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.

- Benign and likely benign variants are not included in this report.

- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University 1-2

اسم الفرع: حدائق، حلوان
مدير الفرع: د. تسون ابراهيم
مدير الوحدة: ا.د. رانيا درويش
طبيب الوحدة: ا.د. عطية الدش

المختبر
al mokhtabar
moamena kamel lab
معامل مومنة كامل

رئيس مجلس الإدارة
ا.د. مؤمنه كامل
مدير المعامل
ا.د. هند الشربيني

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Gender Female		Referred By Prof :-	Client ID 55452

findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.
- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University 2-2

PATIENT NAME صباح سليمان بشاي		Registered 08-05-2026 17:23:08	Collected 08-06-2026 17:24:19
Visit Number 50726508318	Age 63 Year	Authenticated 07-07-2026 15:08:47	Printed 09-07-2026 13:37:24
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Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the *BRCA1* & *BRCA2* genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.
- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, *BRCA2* carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRACA assay
- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of *BRCA1* & *BRCA2* genes.
- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.
- Benign and likely benign variants are not included in this report.
- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University 1-2

PATIENT NAME صباح سليمان بشاي			Registered 08-06-2026 17:23:08	Collected 08-06-2026 17:24:19
Visit Number 50726508318	Age 63 Year	Gender Female	Authenticated 07-07-2026 15:08:47	Printed 09-07-2026 13:37:24
Referred By Prof : -			Client ID 55452	

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.
- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University 2-2

اسم المريض
مدير السرير
مدير الوحدة
طبيب الوحدة
د. ايه المرحلي
د. دعاء خبير
د. فائق رمضان



رئيس مجلس الإدارة
د. د. مؤمنه كامل
مدير المعامل
د. د. هند الشريفي

PATIENT NAME صباح سليمان بشاي			Registered 17-06-2026 15:31:26	Collected 17-06-2026 15:31:45
Visit Number 50726508879	Age 63 Year	Gender Female	Authenticated 18-06-2026 01:47:38	Printed 18-06-2026 21:02:39
Referred By Prof :-			Client ID 467	

HEMATOLOGY REPORT

Test Name	Result	Unit	Reference Range	Previous Result
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Coagulation Profile

Prothrombin Time	10.5	sec	8.2 - 13.1	
Prothrombin Time	11.1	sec		
Control (MPT):	109.3	%	70 - 120	
Prothrombin Concentration	0.97		0.9 - 1.27	
INR				

PT concentration results use calibration curve Log-Log with point-to-Point (not linear) Interpolation.

Sample Type: Citrated Plasma

Method: Optical Clot-Based

INR values should only be used when evaluating patients on oral anticoagulant therapy.

Target Therapeutic Values:

INR 2.0 - 3.0 : Prophylaxis of venous thromboembolism, treatment of venous thrombosis (following heparin therapy), prevention of systemic embolism (atrial fibrillation, valvular heart disease, bioprosthetic heart valves, acute myocardial infarction).

INR 2.5 - 3.5 : Mechanical prosthetic heart valves, recurrent systemic embolism.

Dr.Hala Sheba
Professor of clinical pathology,
Faculty of Medicine, Cairo university

1-1

اسم المريض
مدير الفرج
مدير الوحدة
طبيب الوحدة
د. ايه المرحلي
د. دعاء خيمس
د. فاطم رمضان

رئيس مجلس الإدارة
د. د. مومنة كامل
مدير المعامل
د. د. هناد الشربيني

PATIENT NAME صباح سليمان بشاي			Registered 17-06-2026 15:31:26	Collected 17-06-2026 15:31:44
Visit Number 50726508879	Age 63 Year	Gender Female	Authenticated 18-06-2026 00:45:50	Printed 18-06-2026 21:02:38
Referred By Prof : -			Client ID 467	

HEMATOLOGY REPORT

Test Name

Result

Unit

Reference Range

Complete Blood Picture

Haemoglobin (EDTA Blood)	13.3	g/dL	11.5 - 15.5
Haematocrit (PCV)	43.8	%	36 - 45
RBCs Count (EDTA Blood)	4.87	Millions / cmm	4 - 5.2
MCV	89.9	fL	80 - 100
MCH	27.3	pg	27 - 33
MCHC	30.4	g/dL	31 - 37
RDW-CV	13.7	%	11.5 - 15
Platelet Count	223	Thousands / cmm	150 - 450
Total Leucocytic Count (EDTA Blood)	4.9	Thousands / cmm	4 - 11
<u>Percent Values</u>			
Neutrophils	55.6	%	2 - 7
Lymphocytes	35.4	%	1 - 4.8
Monocytes	7.6	%	0.2 - 1
Eosinophils	0.8	%	0.1 - 0.45
Basophils	0.6	%	0 - 0.1
<u>Absolute Values</u>			
	2.70	x10 ⁹ /L	
	1.72	x10 ⁹ /L	
	0.37	x10 ⁹ /L	
	0.04	x10 ⁹ /L	
	0.03	x10 ⁹ /L	

Comment: NORMAL BLOOD PICTURE.

Dr. Hala Sheba
Professor of clinical pathology,
Faculty of Medicine, Cairo university

1-1

الاسم	6.1	Sabah Soliman	2026/6/14	التاريخ
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Echo report

LVDd	3.5	4.2-5.8cm	FS	25	25-43%
LVSD	2.4	2.5-4cm	EF	58	>55%
IVSd	0.7	0.7-1	AO	2.8	2.6-3.0cm
LVPWd	0.8	0.7-1	LA	3.6	3-4cm

Measurements:

Normal LV dimensions with good systolic function by MM.

- Normal LV wall thickness
- SWMA: Not apparent
- RV wall dimensions, wall thickness, function, TAPSE good
- Aortic root: normal
- LA size: normal
- RA size: Normal
- Septae: Normal

Valves:

- Normal AV morphology and flow
- Normal MV and flow
- Normal TV and flow
- Normal PV and flow
- Pericardium: normal

Intracardiac masses: no

Conclusion: Diastolic dysfunction

Dr Ahmed Maher Msc
Cardiology

Patient ID: 96372	Patient Name English: Sabah Saliman Beshay Yonis
مرضى ٩٦٣٧٢	2025/06/17 10:43:49 AM : Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A 62 -year-old female patient with pathologically proven right breast invasive duct carcinoma grade III (-ve ER, -ve PR, -ve HER2 and Ki67= 60 %).

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Minimal background enhancement

ACR: C (heterogeneously dense breast parenchyma).

Right breast:

- Low energy (Non contrast) images show: Right breast upper outer quadrant irregular shaped indistinct hyperdense mass lesion with internal malignant pleomorphic microcalcifications.
- Recombined (Post contrast) images show:
 - ❖ Corresponding upper outer quadrant heterogeneously enhancing mass lesion (at 10 o'clock) of high conspicuity, with posterior linear non-mass enhancement, measuring overall 3.7x1.6x2 cm, being at about 3.5 cm from the nipple.
 - ❖ Few enhancing nodules are seen posterior to the index lesions with the farthest one being at 2.6 cm from it. By complementary ultrasound, corresponding indeterminate hypoechoic nodules are seen at 11 o'clock.
 - ❖ The overall process measures about 5 cm along its maximum dimension
- Normal skin / nipple / areolar complex.
- An ultrasound guided clip insertion was done in the same setting.

Left breast:

- Low energy (Non contrast) images No masses, asymmetry or distortion or suspicious microcalcifications. Multiple benign looking macrocalcific foci.
- Recombined (Post contrast) images show a retro-areolar enhancing asymmetry of low conspicuity, by ultrasound, corresponding 1 o'clock rather circumscribed oval hypoechoic lesion is noted measuring about 1X0.5 cm.
- Normal skin / nipple / areolar complex.

IMPRESSION:

Right breast: UOQ pathologically enhancing malignant process, as described above (BIRADS 6).

Left breast: UOQ (1 o'clock) probably benign lesion, for ultrasound follow up after 6 months (BIRADS 3).

Best regards,

Dr. Sherihan Fakhr, MD

Dr. Yumna Shaheen, MD



S C A N

Name: Sabah Soliman Beshtay

ID: 5140974
Date: 20/04/2026

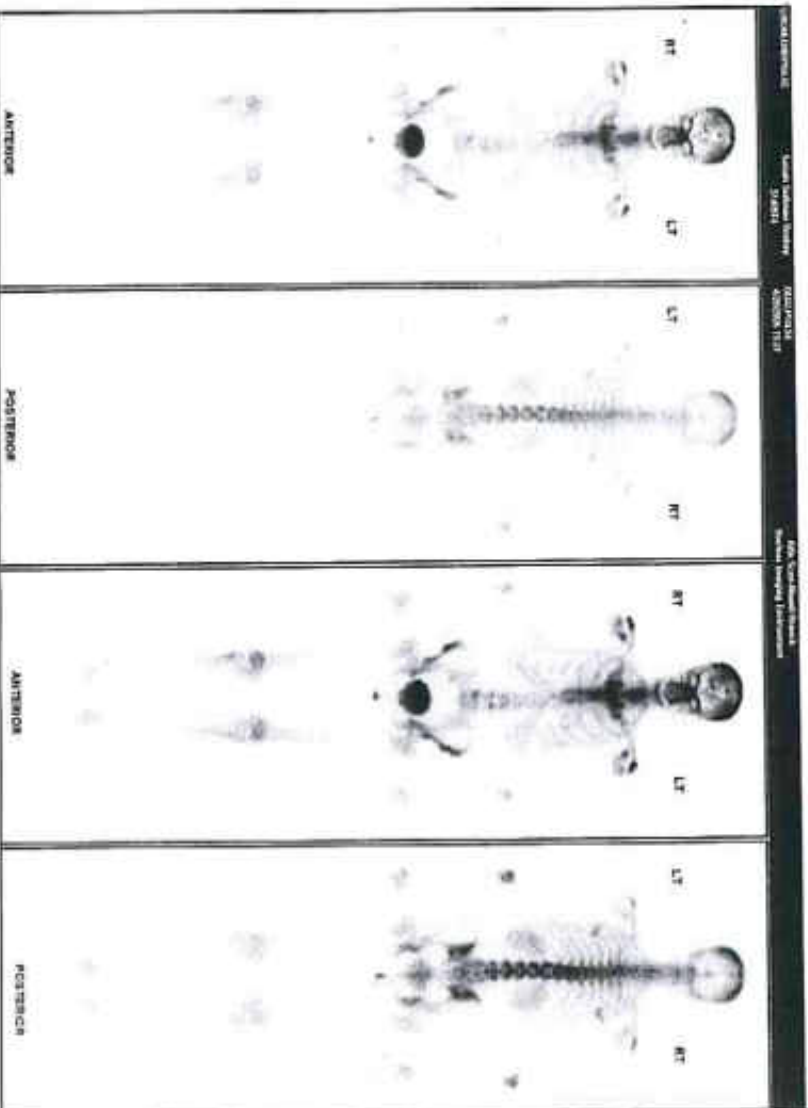
^{99m}Tc -MDP wholebody Bone Scan

Clinical data:

Indication	Right breast cancer.
Tumor markers	No.
Skeletal trauma/ surgeries	No.
Previous imaging	Not available.
Previous bone scan	No.

Technique:

Injected dose	740 MBq
Images	Delayed whole body +spot images of the skull and chest.
Complementary images	No.





**SCANNING
FINDINGS:**

- No abnormal osseous tracer uptake detected that would account for osseous lesions.
 - Arthritic changes of both shoulders are noted.
- Impression:**
- No scintigraphic evidence of metastatic osseous deposits.

Dr. Mona Saied, MD

Dr. Omnia Alaa, MSc

ID-5140974

الفا سكين

AMG

A MEMBER OF ALFA MEDICAL GROUP

114171

Patient ID: 937211	Patient Name English: Sahab Saliman Boshay
المرضى: ١١	2024/03/09 01:37:54 PM Report Date

Women's Health Outreach Program

Bilateral Full Field Digital Mammography

Clinical summary: 53-year old female with positive family history (paternal side grandmother) complaining of right mastalgia.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR c)
- Outer central, middle third, irregular speculated high density mass lesion with obscured margins and related coarse heterogeneous and linear microcalcifications.
- **Ultrasound** demonstrated exaggerated fibroglandular pattern for age with heterogeneous parenchyma showing mottled appearance.
- 9 o'clock, middle third, 3 cm from nipple, 6 mm deep to the skin, irregular anti-parallel heterogeneous hypoechoic solid mass lesion with moderate internal vascularity measuring 23 x 27 mm.
- Anterior ductal extension is seen reaching the nipple through segmental prominent soft tissue echogenicity filled ducts seen reaching 3 mm in caliber.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR c).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated exaggerated fibroglandular pattern for age with heterogeneous parenchyma showing mottled appearance with no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Right breast (BIRADS 4c): Outer central suspicious mass lesion with anterior ductal extension reaching the nipple, US-Guided Tru-cut biopsy was taken at same setting.

Left breast (BIRADS 1): Diffuse breast adenosis.

Recommendations:

For histopathological correlation.

Dr. Amal Hegazi, M.Sc.

92.7

Patient Name : الأستاذ / صباح سليمان بشاي	Request No. : 13005396
Patient No. : 26303212102246	Request Date : 10/03/2026
Path No. : 671/26	Verification Date : 16/03/2026
Sex : Female	Financial Category : جوستاف روسي - مبادرة
Age : 62 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Right breast 9 O'clock mass, (BIRADs 4c).

Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 1:20 pm, Cold ischemia time: 20 seconds.

Gross Examination:

- Three fragmented grey white tissue cores, ranging in size between 0.3 cm & 1.5 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.

Diagnosis:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghonaim.

Verified by :

Dr. Duaa Mohamed



Patient Name	: الأستاذ / صباح سليمان بشاي	Request No.	: 13005396
Patient No.	: 26303212102246	Request Date	: 10/03/2026
Path No.	: 671/26	Verification Date	: 16/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 62 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast 9 O'clock mass, (BIRADs 4c) diagnosed "Invasive duct carcinoma grade 3".
Nature of Specimen: Tru-cut biopsy, collected on 10/3/2026, at 1:20 pm, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 60%.

Conclusion:

Right breast mass, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Low (score 1+).
- Ki67%: 60%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2,

Verified by :

Dr.Duaa Mohamed

Printing Date : 28/03/2026 10:07:59

Page 1 of 2

Printed By : .marwa mohamed yousse



Patient Name الأستاذ / صباح سليمان بشاي
Patient No. 26303212102246
Path No. 671/26
Sex Female
Age 62 Y
Referring Doc.

Request No. 13005396
Request Date 10/03/2026
Verification Date 16/03/2026
Financial Category جوسفاروسي - مبادره
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim.

Verified by :

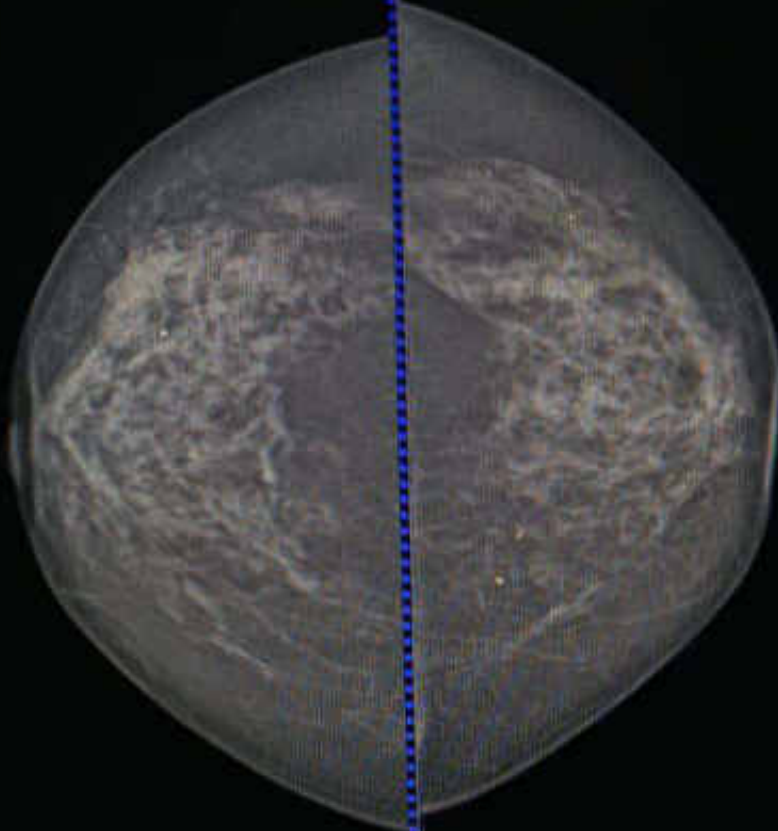
Dr.Duaa Mohamed

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Page 2 of 2

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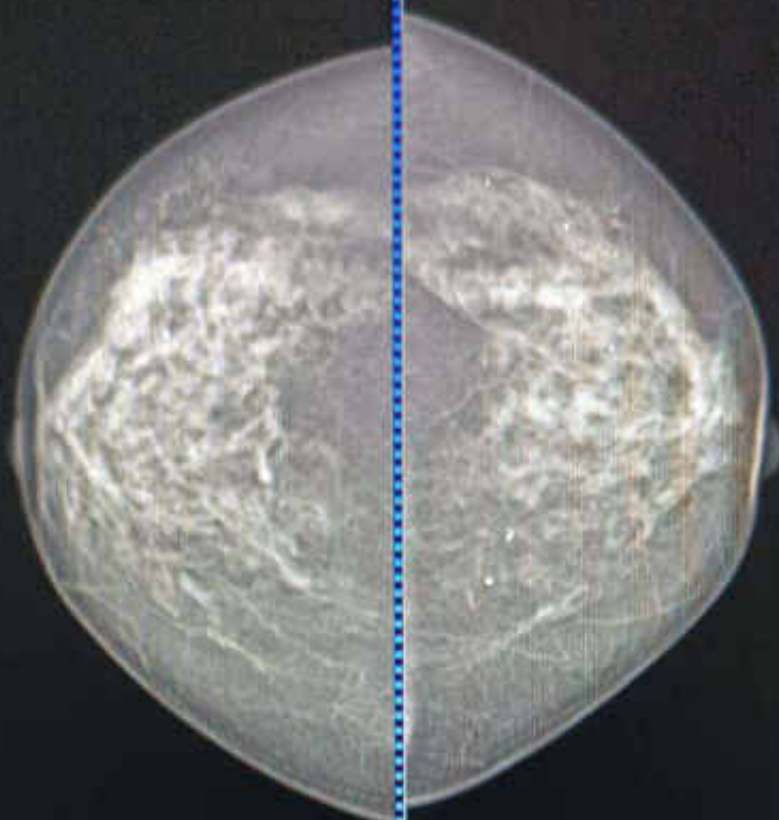
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**GUSTAVE
ROUSSY**
INTERNATIONAL EGYPT

Patient's Name:
Date:
Referred by:
Hospital:

SABAH SOLIMAN BOSHAY BOLIS
2026-04-06
Prof. Dr. CT CAP IV
Gustave Roussy International Egypt

- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticula outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- **Normal CT scan of the abdomen and pelvis.**

Dr Mai Mohamed MD

Dr. Mai Sadiq

Dr. Mai Sadiq

**GUSTAVE
ROUSSY**

Patient's Name:
Date:
Referred by:
Hospital:

SABAH SOLEMAN HOSSEIN BOULEZ
2026-04-06
Prof Dr CT CAP IV
Gustave Roussy International Egypt

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH
CORONAL RECONSTRUCTION:-**

Findings:

- Right breast soft tissue mass measures 17x15 mm
- Few right pectoral lymph nodes measures 11x9 mm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

Right breast soft tissue mass for US correlation.

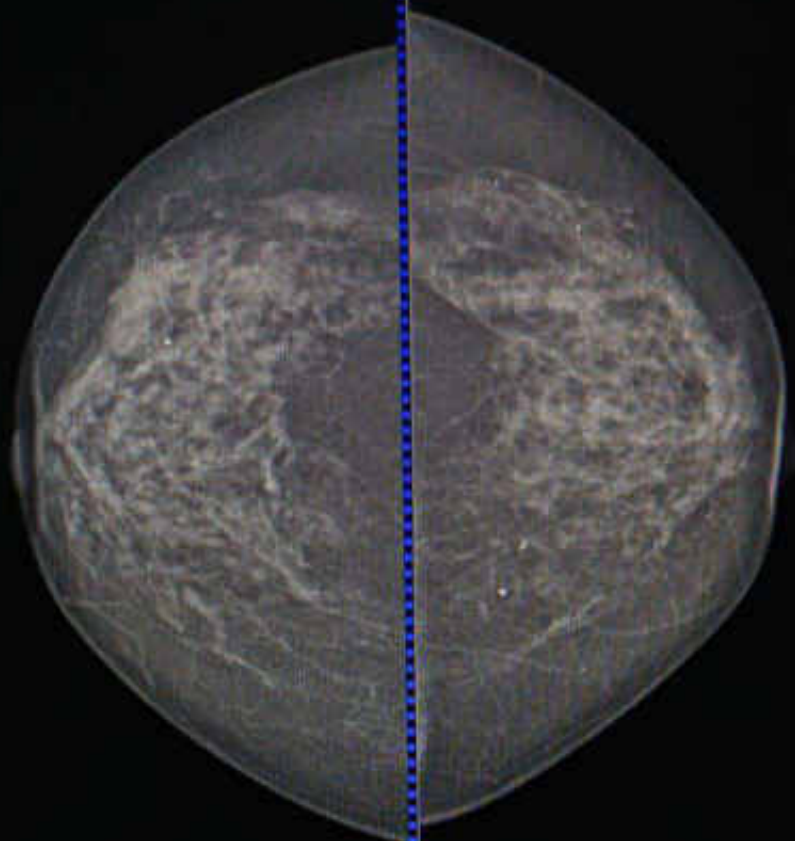
**MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH
ORAL & I.V. CONTRAST ADMINISTRATION:-**

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.

-CC

L-CC



R-MLO

L-MLO

